

Acute Otitis Externa

Patient Group Direction, version 003, issued 9 September 2026. Two products: ciprofloxacin 2mg/ml single-dose ear drops, and dexamethasone with neomycin and acetic acid ear spray.

What this document covers, and what changed

- **THIS IS AN OTITIS EXTERNA SERVICE.** It is not acute otitis media and it does not authorise any oral antibiotic. A patient with otitis media is referred.
- **CIPROFLOXACIN 2MG/ML EAR DROPS ARE NOW AUTHORISED BY THIS DOCUMENT.** Version 001 authorised only the dexamethasone, neomycin and acetic acid spray. Ciprofloxacin drops are the product the consultation tool has been recommending, so the two are now aligned.
- **Otoscopy is a required step and a required competency.** The tympanic membrane must be seen and recorded as intact before either product is supplied.
- **The red flags that sat in the guidance summary are now exclusion criteria in both arms.**
- **ONE COURSE PER EPISODE.** A patient who has not improved after a full course is referred, not re-supplied.

Choosing between the two products

- **Where the tympanic membrane is intact and clearly seen, either product may be used.** Ciprofloxacin drops are preferred where there is any doubt about the drum, because neomycin is an aminoglycoside and is potentially ototoxic if it reaches the middle ear.
- **Ciprofloxacin drops are licensed from 1 year of age.** The spray is licensed from 2 years. For a child aged 1 to under 2, only the drops may be used.
- **Ciprofloxacin drops may be used in pregnancy and while breastfeeding, because systemic exposure after otic administration is negligible.** The spray is not recommended in pregnancy. In a pregnant or breastfeeding woman, use the drops.
- **The spray contains a corticosteroid and may settle a very inflamed, itchy canal faster.** That is its advantage, and the only reason to choose it over the drops.
- **Do not use both.** Do not use any other ear preparation at the same time.

General measures for every patient

- **Keep water out of the ear for the whole course and for a week afterwards.** No swimming. Use a shower cap or petroleum jelly on cotton wool.
- **No cotton buds, no earplugs, no self-cleaning.** This is usually what started it.
- **Paracetamol or ibuprofen for pain, as a separate pharmacy sale subject to the usual checks.** Pain relief matters: otitis externa hurts.
- **Where discharge or debris is obstructing the canal so drops cannot reach the skin, refer for aural toilet.** Do not attempt to clear the canal in the pharmacy.

Ciprofloxacin 2mg/ml ear drops, single-dose containers

Patient Group Direction for the supply of ciprofloxacin 2mg/ml ear drops solution in single-dose containers for the treatment of acute otitis externa in adults and children older than 1 year with an intact tympanic membrane.

This arm is new in version 002. Version 001 did not authorise ciprofloxacin ear drops.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- **ALL USERS MUST BE COMPETENT IN OTOSCOPY.** This PGD cannot be used without examining the ear. The tympanic membrane must be visualised and recorded as intact before either product is supplied, and otitis media, perforation, a grommet, a foreign body and fungal debris must be distinguishable from acute otitis externa.
- Competence in otoscopy must be documented and signed off by the employing organisation before the practitioner works under this PGD. Self-declaration is not sufficient.
- All users must be familiar with the current SPC for the product supplied, the BNF and NICE CKS on otitis externa.
- All users must be able to recognise necrotising (malignant) otitis externa and know that it is an emergency ENT referral, not a treatment failure.
- All users must previously have supplied medicines under a PGD.
- All users must understand capacity and consent, including consent in children and Gillick competence, and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of acute otitis externa in adults and children older than 1 year with an intact tympanic membrane, caused by ciprofloxacin susceptible micro-organisms, in accordance with the current SPC and NICE CKS.
Inclusion criteria	• Aged more than 1 year. There is no upper age limit. • Clinical signs and symptoms of acute otitis externa: ear pain, itch, discharge, a swollen or erythematous canal, or tenderness of the tragus or pinna. • OTOSCOPY PERFORMED, the tympanic membrane visualised, and recorded as INTACT. • No red flag in Appendix 1 present. • No previous course supplied for this episode. • Valid informed consent obtained, from a person with parental responsibility where the patient is under 16 and not Gillick competent. • No exclusion criterion present.
Exclusion criteria: red flags	Refer, do not supply, where ANY of the following is present. These are exclusions, not points to consider. • SEVERE UNREMITTING PAIN IN A PATIENT WITH DIABETES OR WHO IS IMMUNOCOMPROMISED. Suspect necrotising (malignant) otitis externa. This is a same-day emergency ENT referral. It is the one presentation in this document that kills. • Facial nerve palsy, or any weakness or drooping of the face. Same-day emergency ENT referral. • Pain, swelling, redness or tenderness over the mastoid, behind the ear, or an ear that is pushed forward. Suspect mastoiditis. Same-day emergency referral. • Systemic illness: fever, rigors, or the patient appearing unwell. Refer the same day. • Spreading infection: cellulitis of the pinna, the face or the neck. • Vertigo, new hearing loss beyond simple canal blockage, tinnitus of new onset, or any other neurological symptom.

	● Failed treatment after 2 weeks, or failure to improve after a full course under this PGD. Refer; do not supply a second course blind. ● Suspected foreign body in the canal. ● White or black fuzzy debris in the canal, suggesting fungal infection. Antibacterial drops will not treat it and may make it worse. Refer.
Exclusion criteria: all other	Refer, do not supply, where any of the following applies. ● Aged 1 year or under. Safety and efficacy below 1 year have not been established. ● Tympanic membrane not visualised, perforated, or perforation suspected. If you cannot see the drum, you cannot use this PGD. ● Grommet or tympanostomy tube in situ. ● Known hypersensitivity to ciprofloxacin, to any quinolone antibacterial, or to any excipient. ● Otitis media, or any infection requiring an oral antibiotic. This document does not authorise one. ● Chronic otitis externa, meaning symptoms lasting more than 3 months. Refer to ENT. ● Recurrent otitis externa: 3 or more episodes in the last 12 months. Refer to ENT for investigation. ● Already using another ear preparation. Concomitant ear preparations are not recommended. ● Valid informed consent not obtained.
Cautions	Discontinue at the first sign of a skin rash or any other sign of hypersensitivity. Serious and occasionally fatal anaphylactic reactions, some after a first dose, have been reported with systemic quinolones. Warm the ampoule in the hand for a few minutes before use. Instilling cold solution into the ear causes dizziness. Use may allow overgrowth of non-susceptible organisms including yeasts and fungi. If the patient returns worse with fuzzy white or black debris, that is a fungal superinfection: refer, do not repeat. If signs and symptoms persist after one week of treatment, further evaluation is needed. Refer. For auricular use only. Not for the eye, not for inhalation, not for injection.
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Where a red flag is present, refer the same day, and for suspected necrotising otitis externa, facial palsy or mastoiditis, arrange emergency ENT assessment rather than a routine GP appointment. Give pain relief advice and the water-avoidance advice regardless of whether anything is supplied. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Ciprofloxacin 2mg/ml ear drops solution in single-dose container (for example Cetraxal). Each 0.25mL ampoule delivers 0.5mg ciprofloxacin.
Legal category	POM.
Dose and frequency	The contents of one single-dose ampoule instilled into the affected ear twice daily for 7 days. Where an otowick or tampon is used to aid administration, the FIRST dose only is doubled, using two ampoules.
Quantity to be supplied	One pack of 15 single-dose ampoules. A 7 day course at twice daily uses 14 ampoules, with one spare. Supply for one ear unless both are affected and both were examined. No repeat supply under this PGD.

Maximum treatment period	7 days. One course per episode. A patient not improving at the end of the course is referred, not re-supplied.
Route and method of administration	• Warm the ampoule by holding it in the hand for several minutes. • The patient lies with the affected ear upward. Instil the drops, then pull gently on the auricle several times. • Stay in that position for about 5 minutes to let the drops penetrate. Repeat for the other ear if both are affected. • Discard the ampoule after use. Do not keep it for the next dose.
Storage	Store below 30 degrees Celsius, in the original packaging to protect from light. Once the foil pouch is opened, the remaining ampoules must be used within 8 days.
Drug interactions	No interaction studies have been performed. Systemic absorption after otic use is negligible, so systemic interactions are unlikely. Do not use other ear preparations at the same time.
Pregnancy and breastfeeding	May be used in both. Systemic exposure after otic administration is negligible and no effects are anticipated (SPC section 4.6). This is the arm to use in a pregnant or breastfeeding woman.
Adverse effects	Uncommon: ear pruritus, tinnitus, dizziness, headache, dermatitis, application site pain. Very rare with locally applied fluoroquinolones: generalised rash, toxic epidermal necrolysis, exfoliative dermatitis, Stevens-Johnson syndrome, urticaria.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and from whom. Where the patient is a child, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • THE OTOSCOPY FINDING, in terms: which ear, the state of the canal, and that the tympanic membrane was seen and was intact. Where the drum could not be visualised, that fact and what was done about it. • That every red flag in Appendix 1 was asked about or looked for, and was absent. • Diabetes and immunosuppression status, and the severity of pain. • How long symptoms have been present, and any treatment already tried. • Any previous episode in the last 12 months, and any previous course supplied under this PGD. • Which product was supplied and why. • Name, form, strength, dose, quantity and date of supply. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Dispose of used ampoules and any unused product in accordance with local requirements. Advise the patient to return unused ampoules to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• Warm the ampoule in your hand first. Cold drops make you dizzy. • Lie with the sore ear facing up, put the drops in, tug the ear gently a few times, and stay there for 5 minutes. • Twice a day for 7 days. Use a new ampoule each time and throw it away afterwards. • Keep water out of the ear for the whole course and a week after. No swimming. • Nothing goes in the ear: no cotton buds, no earplugs. • Paracetamol or ibuprofen for the pain if they suit you.
Follow-up and when to get help urgently	Seek help the SAME DAY if any of these happen: • The pain becomes severe and does not let up, particularly if you have diabetes or a weakened immune system. • Any weakness or drooping of your face. • Swelling, redness or pain in the bone behind your ear, or your ear starts to stick out. • You develop a temperature or feel generally unwell. • A rash, or any swelling of the lips, face or throat. Stop the drops. Otherwise, if you are no better after finishing the 7 days, go to your GP. Do not start a second course.

Dexamethasone with neomycin sulfate and acetic acid ear spray

Patient Group Direction for the supply of dexamethasone 0.1% w/w, neomycin sulfate 0.5% w/w and acetic acid 2% w/w ear spray for the treatment of mild to moderate acute otitis externa in adults and children aged 2 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- **ALL USERS MUST BE COMPETENT IN OTOSCOPY.** This PGD cannot be used without examining the ear. The tympanic membrane must be visualised and recorded as intact before either product is supplied, and otitis media, perforation, a grommet, a foreign body and fungal debris must be distinguishable from acute otitis externa.
- Competence in otoscopy must be documented and signed off by the employing organisation before the practitioner works under this PGD. Self-declaration is not sufficient.
- All users must be familiar with the current SPC for the product supplied, the BNF and NICE CKS on otitis externa.
- All users must be able to recognise necrotising (malignant) otitis externa and know that it is an emergency ENT referral, not a treatment failure.
- All users must previously have supplied medicines under a PGD.
- All users must understand capacity and consent, including consent in children and Gillick competence, and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of mild to moderate acute otitis externa in adults and children aged 2 years and over with an intact tympanic membrane, in accordance with the current SPC and NICE CKS.
Inclusion criteria	• Aged 2 years and over. There is no upper age limit. • Clinical signs and symptoms of mild to moderate acute otitis externa. • OTOSCOPY PERFORMED, the tympanic membrane visualised, and recorded as INTACT. • No red flag in Appendix 1 present. • No previous course supplied for this episode. • Not pregnant. Use the ciprofloxacin arm instead. • Valid informed consent obtained, from a person with parental responsibility where the patient is under 16 and not Gillick competent. • No exclusion criterion present.
Exclusion criteria: red flags	Refer, do not supply, where ANY of the following is present. These are exclusions, not points to consider. • SEVERE UNREMITTING PAIN IN A PATIENT WITH DIABETES OR WHO IS IMMUNOCOMPROMISED. Suspect necrotising (malignant) otitis externa. This is a same-day emergency ENT referral. It is the one presentation in this document that kills. • Facial nerve palsy, or any weakness or drooping of the face. Same-day emergency ENT referral. • Pain, swelling, redness or tenderness over the mastoid, behind the ear, or an ear that is pushed forward. Suspect mastoiditis. Same-day emergency referral. • Systemic illness: fever, rigors, or the patient appearing unwell. Refer the same day. • Spreading infection: cellulitis of the pinna, the face or the neck. • Vertigo, new hearing loss beyond simple canal blockage, tinnitus of new onset, or any other neurological symptom. • Failed treatment after 2 weeks, or failure to improve after a full

	course under this PGD. Refer; do not supply a second course blind. • Suspected foreign body in the canal. • White or black fuzzy debris in the canal, suggesting fungal infection. Antibacterial drops will not treat it and may make it worse. Refer.
Exclusion criteria: all other	Refer, do not supply, where any of the following applies. • Aged under 2 years. Use the ciprofloxacin arm from 1 year, or refer. • Tympanic membrane not visualised, perforated, or perforation suspected. Neomycin is an aminoglycoside and is potentially ototoxic if it reaches the middle ear. If you cannot see the drum, use the ciprofloxacin arm or refer. • Grommet or tympanostomy tube in situ. • Known hypersensitivity to neomycin, to any aminoglycoside, to dexamethasone, to acetic acid or to any excipient. • Pregnancy. This product is not recommended in pregnancy; use the ciprofloxacin arm. • Breastfeeding, unless a decision has been made and recorded about whether to continue breastfeeding or the treatment. The ciprofloxacin arm avoids the question. • Otitis media, or any infection requiring an oral antibiotic. This document does not authorise one. • Chronic otitis externa, meaning symptoms lasting more than 3 months. Refer to ENT. • Recurrent otitis externa: 3 or more episodes in the last 12 months. Refer to ENT. • Already using another ear preparation. • Valid informed consent not obtained.
Cautions	Confirm the tympanic membrane is intact before use. This is the single most important check for this product. Avoid prolonged use, to reduce the risk of sensitisation to neomycin and of resistance. Neomycin sensitisation is common and lasts a lifetime. Shake the bottle well before use and advise the patient to keep water out of the ear. Refer if there is no improvement within 7 days, if symptoms worsen, or if irritation or rash occurs. Visual disturbances have been reported with corticosteroid use. Refer to an ophthalmologist for evaluation where a patient reports blurred vision or other visual disturbance.
Actions if the patient is excluded or declines	As for the ciprofloxacin arm. Where the exclusion is pregnancy, breastfeeding, an uncertain drum or an age below 2 years, consider the ciprofloxacin arm rather than referring, provided its own criteria are met.

The medicine

Name, form and strength	Dexamethasone 0.1% w/w, neomycin sulfate 0.5% w/w and acetic acid 2% w/w ear spray.
Legal category	POM.
Dose and frequency	One metered spray into the affected ear three times daily, in accordance with the current SPC. Prime the pump before first use.
Quantity to be supplied	One bottle. No repeat supply under this PGD.
Maximum treatment period	7 days, extended to a maximum of 14 days only where the patient has clearly improved but not resolved and the ear has been re-examined. One course per

	episode.
Route and method of administration	Topical spray into the external auditory canal. Shake well before use. Keep the head tilted, or lie on the side, for a few minutes after applying.
Storage	Store below 25 degrees Celsius. Do not refrigerate or freeze. Store upright.
Drug interactions	Do not use other ear preparations at the same time. Refer to the current SPC.
Adverse effects	Local irritation, stinging or burning on application, pruritus, and hypersensitivity reactions including contact sensitisation to neomycin. Visual disturbance has been reported with corticosteroid use.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and from whom. Where the patient is a child, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • THE OTOSCOPY FINDING, in terms: which ear, the state of the canal, and that the tympanic membrane was seen and was intact. Where the drum could not be visualised, that fact and what was done about it. • That every red flag in Appendix 1 was asked about or looked for, and was absent. • Diabetes and immunosuppression status, and the severity of pain. • How long symptoms have been present, and any treatment already tried. • Any previous episode in the last 12 months, and any previous course supplied under this PGD. • Which product was supplied and why. • Name, form, strength, dose, quantity and date of supply. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Advise the patient to return any unused product to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• Shake the bottle well. One spray into the sore ear, three times a day. • Keep your head tilted or lie on your side for a few minutes afterwards. • Keep water out of the ear for the whole course and a week after. No swimming. • Nothing goes in the ear: no cotton buds, no earplugs. • Stop and tell us if the ear becomes more irritated or you get a rash. Some people become allergic to neomycin.
Follow-up and when to get help urgently	Seek help the SAME DAY for severe unrelenting pain (especially with diabetes or a weakened immune system), any facial weakness or

drooping, swelling or pain in the bone behind the ear, or a temperature.

If you are no better after 7 days, go to your GP. Do not start a second course.

Appendix 1: Red flags. Ask and look for these before supplying anything

Any one of these excludes. The first three are emergencies.

Severe unremitting pain in a patient with diabetes or immunosuppression	• Suspect necrotising (malignant) otitis externa. • EMERGENCY ENT REFERRAL the same day. Needs imaging and intravenous antibiotics. • Most common in older patients with diabetes. Pain out of proportion to the appearance of the canal is the clue.
Facial nerve palsy	• Weakness or drooping on one side of the face. • EMERGENCY ENT REFERRAL the same day.
Mastoid signs	• Pain, swelling, redness or tenderness behind the ear, or a pinna pushed forward and down. • Suspect mastoiditis. EMERGENCY REFERRAL the same day.
Systemic illness	• Fever, rigors, or the patient looks unwell. • Refer the same day.
Spreading infection	• Cellulitis of the pinna, face or neck. • Refer the same day.
Failed treatment	• No improvement after 2 weeks, or after a full course under this PGD. • Refer. Do not supply a second course.
Fungal debris	• White or black fuzzy debris in the canal. • Antibacterial treatment will not help and may worsen it. Refer.
Foreign body	• Refer for removal.
Drum not visualised, perforated, or grommet in situ	• Do not supply the neomycin spray under any circumstances. • Refer.
Recurrent or chronic	• 3 or more episodes in 12 months, or symptoms over 3 months. • Refer to ENT for investigation.

Appendix 2: Key references

- NICE Clinical Knowledge Summary: Otitis externa.
- Summary of Product Characteristics for ciprofloxacin 2mg/ml ear drops solution in single-dose container, and for dexamethasone with neomycin sulfate and acetic acid ear spray, current versions.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>

Authorisation

Version	v003
Supersedes	Version 002, 8 September 2026
Valid from date	9 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health
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	GMC: 6047293 Signed: N. Shori Date: 9 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v003
Date	8 September 2026
Changes	• The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • Ciprofloxacin 2mg/ml single-dose ear drops added as a second, and preferred, arm. Version 001 authorised only the dexamethasone, neomycin and acetic acid spray, while the consultation tool for this service recommends ciprofloxacin drops. The document now authorises the medicine that is actually being supplied. • Ciprofloxacin drops are preferred where the tympanic membrane cannot be clearly seen, in pregnancy and while breastfeeding, and in children aged 1 to under 2, none of which the spray covers. • Otoscopy is now a required step and a required, separately signed-off competency. The tympanic membrane must be visualised and recorded as intact before either product is supplied. • The red flags that previously appeared only in the guidance summary are now exclusion criteria in both arms, in Appendix 1. • Severe unremitting pain in a patient with diabetes or immunosuppression is an explicit exclusion and an emergency ENT referral, being the presentation in which necrotising otitis externa is missed. • Mastoiditis added as a red flag and exclusion. It was not mentioned anywhere in version 001. • Fungal debris, foreign body, spreading infection and new neurological symptoms added as exclusions. • One course per episode, with a referral trigger rather than a repeat. Recurrent otitis externa is defined as 3 or more episodes in 12 months and is referred to ENT. • Records must now carry the otoscopy finding in terms, the red flag check, diabetes and immunosuppression status, and any previous episode in the last 12 months. • Aural toilet clarified as a referral, not something to attempt in the pharmacy.

Previous versions

001	Development and issue of new PGD. Dexamethasone, neomycin sulfate and acetic acid ear spray only, ages 2 and over.
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Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Premises name
 Address
 Postcode:
 ODS code, if applicable

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

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